



Internal Medicine Residency GME

Policy & Progressive Autonomy Framework

2025 - 2026

This policy is made as adjunct to previous policies. Its purpose is to provide clear and concise rules and guidelines for residents with regards to floor, ICU rotations and scheduling. If certain situations are not mentioned in the following policies, please reach out to the current chief residents and/or program director for guidance.

SUPERVISION OF RESIDENTS

Each patient on the Internal Medicine Resident Teaching Service has an identifiable, and privileged attending physician (or licensed independent practitioner) who is ultimately responsible for that patient's care. This information is available to residents, faculty members, and patients. Residents and faculty members inform patients of their respective roles in each patient's care.

Supervision of the residents is exercised through a variety of methods. Some activities require the physical presence of the supervising faculty member. For many aspects of patient care, the supervising physician may be a more advanced resident or fellow. Other portions of care provided by the resident can be adequately supervised by the immediate availability of the supervising faculty member or resident physician, either in the institution, or by means of telephonic and/or electronic modalities. In some circumstances, supervision may include post-hoc review of resident-delivered care with feedback as to the appropriateness of that care.

LEVELS OF RESIDENT SUPERVISION

To ensure oversight of resident supervision and graded authority and responsibility, the program uses the following classification of supervision:

- **Direct Supervision** – the supervising physician is physically present with the resident and patient.
- **Indirect Supervision:**
 - With direct supervision immediately available – the supervising physician is physically within the hospital or other site of patient care, and is immediately available to provide Direct Supervision.
 - With direct supervision available – the supervising physician is not physically present within the hospital or other site of patient care, but is immediately available by means of telephonic and/or electronic modalities, and is available, if necessary, to provide Direct Supervision.
- **Oversight** – The supervising physician is available to provide review of procedures/encounters with feedback provided after care is delivered.

The program director and faculty members must assign the privilege of progressive autonomy and responsibility, conditional independence, and a supervisory role in patient care delegated to each resident. The program director will evaluate each resident's abilities based on specific criteria as outlined. Faculty members functioning as supervising physicians will delegate portions of care to residents, based on the needs of the patient and the skills of the residents.

Senior residents or fellows will serve in a supervisory role of the first year residents in recognition of their progress toward independence, based on the needs of each patient and the skills of the individual resident or fellow. Each resident must know the limits of his/her scope of authority, and the circumstances under which he/she is permitted to act with conditional independence.

Guidelines for circumstances and events in which residents must communicate with appropriate supervising faculty members, such as the transfer of a patient to an intensive care unit, or end-of-life decisions are outlined.

In particular, PGY-1 residents will be supervised either directly or indirectly with direct supervision immediately available when necessary.

CLINICAL RESPONSIBILITIES

The clinical responsibilities for each resident must be based on PGY-level, patient safety, resident education, severity and complexity of patient illness/condition and available support services.

1) First-year residents

- a) PGY-1 residents are responsible for day-to-day care of patients assigned to them. They perform initial history and physicals, follow up examinations and write or dictate discharge summaries for their patients. They also are required to collect all the

necessary data for their patients, update their senior residents and/or attending, and prepare handoffs daily.

b) Any changes in clinical status of patients obtained by PGY-1 residents are to be communicated to the senior residents assigned to the teams and attending physicians who supervise them.

c) PGY-1 residents are expected to present patients to Attendings during rounds with gathered information, including history provided by patients, family or others, physical examination and laboratory data. The information gathered is to be presented to the team as in written notes: following the SOAP format. (SOAP= Subjective, Objective, Assessment and Plan)

d) PGY-1 residents are responsible for examining all follow-up patients prior to rounds-preferably before morning report. New patients are to be seen promptly after morning report, which ends by 8:15 AM, in order to be ready for rounds which typically begins around 9:30-10:30 AM depending on the unit.

2) Senior (second and third year) residents

a) PGY-2 and PGY-3 residents are responsible for: 1) Assuring that attendings are fully supported and integrated into the medical care team; 2) Assuring patients receive high-quality care, and 3) that junior residents receive high-quality education and supervision.

b) Senior residents are responsible for serving as the liaison between attending physicians and resident team members. They communicate regularly with respective attending physicians whose patients are treated by the team and organize work rounds.

c) Senior residents are primarily responsible for supervision and teaching of the first year residents. The senior resident is ultimately responsible for the performance of first year residents and must therefore aggressively seek to assure that patient management decisions by those team members are timely and appropriate. In general, this supervision will be exercised through direction of first-year residents, guiding them in evaluation and management.

d) Specifically, senior residents are expected to: 1) Organize the work of the team so that it is equitably distributed among first-year residents; 2) assist interns in management of patients, to assure that plans and orders are appropriate 3) assure interns are appropriately supervised in performing procedures; 4) assure patients receive high-quality care, and 5) review and/or addend PGY-1 history and physicals and progress notes.

e) Senior residents are responsible for assuring that adequate teaching occurs on the service. They are required to organize formal team activities, including 1) daily work rounds with all residents 2) daily reviews of all patients on the service with all team members, and 3) regular team reviews of radiological imaging, pathology specimens,

and other important materials. The senior resident may organize special teaching sessions as appropriate.

f) On some rotations, PGY-2 and PGY-3 residents may have limited primary patient-care responsibilities. Those activities do not absolve a senior resident from the responsibility of supervising first year residents.

Residents are expected to deal with all aspects of their patients' needs, including being the primary source of information and counseling for patients and their families, assuring appropriate long-term care, completing insurance and other legal forms, etc.

	QUARTER 1 (JULY TO OCT)	QUARTER 2 (NOV- FEB)	QUARTER 3 (MARCH TO JUNE)
PGY1	- Gather history, thorough physical examination, and data/ convey all information to senior and attending 3-4 patients initially with expected increase in patient load and by the end of the quarter should be able to carry 10 patients - supervised entry of orders	- Gather history and physical, pertinent information/ convey to senior and attending - patient load should be 10 - supervised communication with consultants - unsupervised entry of orders	- gather history and physical, pertinent information/ convey to senior and attending - patient load should be 10 - unsupervised communication with consultants - unsupervised entry of orders
PGY2	- Oversees all aspects of patient care - ensures orders are entered and supervises entry of orders - relay all information to attendings, especially changes in clinical status - communicates with consultants	- Oversees all aspects of patient care - ensure orders are entered - relay pertinent information to attendings, especially changes in clinical status - communicates with consultants	- Oversees all aspects of patient care - ensure orders are entered - relay pertinent information to attendings, especially changes in clinical status - communicates with consultants if pgy1 unable to do so - coaches intern on how to be a second year

PGY3	· Oversees all aspects of patient care · ensures orders are entered and supervises entry of orders · relay all information to attendings, especially changes in clinical status · communicates with consultants	· Oversees all aspects of patient care · ensure orders are entered · relay pertinent information to attendings, especially changes in clinical status · communicates with consultants	· oversees all aspects of patient care · ensure orders are entered · relay pertinent information to attendings, especially changes in clinical status · communicates with consultants if pgy1 unable to do so · coaches intern on how to be a second year
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PATIENT CARE

History and Physical

Residents who are assigned a new patient on floor rotations should examine the said patient within 2 hours of assuming care. In the event that this cannot be done, basic orders should be placed on the chart. This document should be completed before midnight on the same day after seeing the patient. Primary care physicians should be notified once the patient is admitted, whenever possible.

Follow Ups

All notes for follow up patients should be completed by midnight on the day of service. This note should include subjective history obtained from patients, nurses and other allied health providers. The goal of the resident should be to have all notes completed by 5 pm, however note-writing should not supersede patient care/safety. Follow up notes should be signed prior to leaving for the day.

In an instance where a patient is discharged on the same day, it is at the discretion of the team attending if a full discharge summary including physical exam will suffice instead of a progress note.

Discharges

Medication reconciliation should be completed in anticipation of a patients' discharge prior to rounds to allow the team attending to double check the medication list.

Prescriptions may be signed by residents but have to be reviewed by the attending ideally the day prior to the planned discharge or before noon the day of discharge. If the patient is from our clinic, a clinic appointment with the junior or senior resident should be arranged. Private patients are to be instructed to follow-up with their primary care physician.

Discharge note should be completed within 24 hours of discharge, ideally the same day of the patient's discharge. This is extremely important as most patients are encouraged to see their PCP's within the same week of discharge, and by having the discharge summary available will assist in their continuity/transition of care. It is at the discretion of the team attending if a full

discharge summary including a physical exam will suffice instead of a progress note on the day of the patient's discharge.

Consultations

It is the intern/resident's responsibility to check prior admissions and consult the specialist who has seen the patient in the past. If no consultants have seen the patient in the past, designated consultant on the monthly schedule will be placed. No exceptions unless directed by attending.

In the event a consult needs to be cancelled for whatever reason the intern/resident should:

1. Call the consultant and inform them of the cancellation.
2. Call the charge nurse for whatever floor the patient is on and let them know the consult has been cancelled.

STAT Consults

In critical situations, STAT consults will be performed by the senior resident. All STAT consults must be called in a timely fashion, so that decisions regarding the patient's care are done. The consultant is to be informed for the STAT consult prior to entering it in Meditech. When placing STAT consults into the EMR, please mark "Yes" to physician notification and document the time and name of the physician the case was discussed with.

Code Blues and Rapid Response

All ICU residents are to respond to ALL code blue regardless of the attending on file. No exceptions. Interns will be responsible for compressions and if intensivist is not present, seniors will run the code.

In regards to rapid responses, if it is a patient on the hospitalist service, residents on rapids call or STAR/Night residents are to attend. Senior residents are responsible for running the rapid response and notifying the on-call attending of the situation. A rapid response note must be documented in Meditech after the code.

Rotations Schedules, Caps and Expectations

Each rotation has a different schedule with different expectations. This policy will outline just the pertinent items, however this is subject to change at the discretion of the Chief Residents and Program Director.

Floor Rotations:

1. Arrive on time- no later than 6 am.
2. Await patient distribution and receive signout from night team
3. Send list of patients to the respective attending on the team
4. Log into MH Cure (I-mobile) at the beginning of your shift
5. Pre-chart your patients until morning report.

6. Pre Round on your patients- use your time wisely. Make sure all patients are seen prior to rounds. Communicate with the team any changes in status or tardiness to rounds.
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Team CAPS:

Floor Teams	End of July	End of August	End of September
3 Seniors	30	30	
1 Senior + 2 Interns	14	18	20
1 Senior + 1 Intern	12	14	14

- Note: If there are two seniors (two PGY-2 or one PGY-2 with one PGY-3) on a team, each senior will be responsible for supervising an intern for up to 10 patients. There will be no exceptions to this policy.

Intern floor patient Progression:

	*Intern	Senior
First 2 Weeks of July	Min 3- Max 5	12
Last 2 Weeks of July	Min 4- Max 6	14
First 2 Weeks of August	Min 6- Max 8	16
Last 2 Weeks of August	Min 8- Max 10	18
September	Max 10	20

- *Note: These are the **minimum** number of patients required to be seen daily for the intern. Intern is permitted to see up to 2 more patients than the minimum requirement. This will be up to the discretion of the attending, senior and intern.
- Senior caps above are for 1 senior with 2 interns.

July:

- Starts off with a minimum of 3 patients per day. The cap will increase as above every 2 weeks. Minimum number of patients will be 4 patients by the end of the month. Interns should be placing admission orders, doing medication reconciliation under **DIRECT** Supervision of the senior resident. Interns may have a maximum of 2 H&Ps in a 24 hour period.

August:

- Minimum increases to 6 Patients per day and by the end of the month goes to 8 patients. Interns may have a maximum of 4 H&P's in a 24 hour period. During this time, they may call case managers.

After the CCC meets in late December, it is not uncommon for the intern to see up to 10 patients without a senior resident.

Senior residents can be assigned up to 12 patients (PGY 2) and 14 patients (PGY 3) until the CCC meets mid year if working independently. Once the CCC meets, the cap may increase to ensure progress to independent practice. The increase in cap is to ensure that by the end of the residency, you are able to work independently successfully. *The ACGME provides no caps limitations for senior residents who work independently. However the census is adjusted based on patient acuity, support staff, duty-hour compliance and resident feedback to ensure patient safety and educational value.*

Intern Night Float Cap:

July: No more than 2 H&P's overnight.

August: No more than 4 H&P's overnight and will remain the cap for the rest of the academic year per ACGME.

Note:

1. Senior cap on night float is 10 H & P's throughout the academic year if 1 senior with 2 interns. If one senior is supervising one (1) intern, the cap is 8.
2. While on nights, patient distribution is up to the senior's discretion
3. NOTE: PGY-2/3 residents scheduled for Night Float during the month of July are required to oversee all PCU patients. PGY-3 residents will not be supervising PGY-2 residents, however will be available if they need assistance. PGY-2/3 residents are required to go to all Rapids/Code-Blue's on nights.

Direct supervision will occur until October 31st. After which first year residents can work independently of the senior residents.

Senior residents are able to admit up to 6 patients if PGY2 or 8 patients if PGY 3 without interns.

ICU Expectations:

1. Arrive on time for sign out no later than 6:00 am for day shift or 6:50 pm for night shift
2. Promptly start seeing patients and reviewing chart upon conclusion of sign out
3. Start notes on respective pts as soon as possible- initiate note and place them on pending status (ie save)
4. Sign in to MH-Cure (li-mobile) at start of shift
5. Always be readily available on unit
6. Be fully engaged, learn how to place US guided IVs, NGT, Foleys, arterial lines, central lines, intubations.
7. Make good use of any down time by reading up on the patient's disease processes.
8. Round on patients hourly and prn based on clinical context
9. Examine and personally evaluate patients at bedside when informed of changes by nurse. Then notify attending of assessment.
10. All orders (labs/meds/imaging) in ICU are generally placed STAT unless otherwise indicated by attending (consults are per attending instruction)
10. Seniors to supervise all new Intern order entry

Closed loop communication is imperative in the ICU.

ICU CAP Progression:

ICU Teams	End of July	End of August	End of September
1 Senior + 1 Interns	12	15	16
1 Senior + 2 Interns	14	15	16

Our ICU teams typically assign 12-16 patients to a senior-intern team, however it is adjusted dynamically based on patient acuity, staffing and duty-hour compliance. Census is reduced when complexity and acuity increases. The attending physicians are on site 24-7, and work closely with the resident team to supervise. When complexity and acuity increase, the attending physician supervises the intern individually/directly, offloading that workload from the senior resident.

	*Intern
First 2 Weeks of July	2-3
Last 2 Weeks of July	3-4
First 2 Weeks of August	4-5
Last 2 Weeks of August	5-6
September	7

* NOTE: This will be up to the mutual decision of the intensivist, senior and intern.

Seniors working independently can see up to 10 patients until the CCC meets mid-year to re-evaluate the CAPs.

After the mid-year CCC, we may allow interns to work independently from senior residents.

All orders placed by first year residents are to be directly supervised by the senior resident in the first 3 months. No exceptions.

Procedures:

Residents performing diagnostic procedures that require a high level of expertise in performance or interpretation will receive **General, Direct, or Personal supervision** by a faculty member, depending on the experience and proficiency previously demonstrated by the resident.

Procedures within the ICU are typically distributed according to the intern/resident taking care of the patient.

- If the senior resident is not certified in a particular procedure, then they will take precedence before interns in performing the procedure until that senior is certified.
- During codes interns will not be able to perform intubations or central lines unless indicated by the intensivist.
- Interns cannot run "Code-Blues", this will be left for PGY-2 and PGY-3's.
- As with any other rotation, the attending/intensivist can overrule the senior's decision.

Rapid Response/Clinical Status Changes:

1. Ward Interns and their respective supervising senior residents must personally evaluate a patient when called for a clinical status change, review the medical record and meds that patient is currently on.
2. Initiate/order appropriate labs and work up if no recent labs/imaging available in EMR.
 - for example: CBC/coags/chemistry/ABG/lactate/cultures/CXR etc. as time/clinical situation allows
 - work collaboratively with ICU charge nurse and head recommendations to escalate when necessary for crashing patients
3. Speak to their respective supervising Attending for further instructions
4. Consult ICU when requested by their supervising Attendings
5. Senior Residents (PGY 2/3) are expected to present the case directly to the ICU attending
6. Wait for ICU team instruction/evaluation before moving the patient to ICU.

It is imperative that residents stay with patients until clinically stable or physically moved to the ICU or when an intensivist arrives at bedside to evaluate.

Morning Report:

Signouts are to be done before Morning report. Morning report takes place Monday-Friday Morning report runs from 7:15 AM to 8:15AM. Attendance will be recorded for Morning Report. All wards and night shift residents are required to be present.

*If you are running late or absent, please let one of the Chief Residents know

Any problems that arise on the medical service and other acute issues that may affect the medical service may be presented and discussed. All residents may present clinical problems at that time that may affect the care of their patients and especially anticipated problems that may arise for the on-call team. **It is expected that the overnight intern will present an overnight admission on the first week of the block rotation.** Typically the cases will be chosen by the overnight senior resident if not previously determined by the attending.. Cases will be discussed in detail along with a Chief Resident and Attending who will be present.

If an attending requests a certain case to be presented, that case must be presented regardless of the case prepared by overnight or floor team. NO EXCEPTIONS.

Morning Rounds:

Morning rounds typically start at 9:00-9:30 am, depending on the team and last until 12 pm. It is the team senior's responsibility to ensure that rounds finish prior to 12 pm, so that the team can have time to get lunch and make it on time for Noon Conference.

Afternoon Conference:

Afternoon conference starts at **12 pm** on Mondays-Wednesday & Fridays. It starts at 12:30pm on Fridays. It is expected that everyone shows up on time. Attendance is MANDATORY. If you are running late, please have your respective attending message one of the chief residents. It is expected of every resident to show up on time, as this is a professional courtesy of the presenter and attendees.

Afternoon conference is protected time. Between the times of 12-1:00pm, the use of personal devices will be prohibited to ensure education remains a priority. Nursing staff will be informed to divert all calls to the attendings during that time. Only off site rotations are excused from the afternoon conference.

Evening Sign Out:

Daily Hand-offs typically take place in the Resident Work Room around 7:00 pm for floor teams & night teams. Residents must prepare a sign-out sheet which includes the patients' names, room numbers, diagnoses, attendings, status, and complications. The full on-call team must be present for sign out.

Patient Keeper is available and the method used for patient census signout and should be updated daily and printed to be used for sign-out. Issues that require follow-up **must** be written down along with the appropriate action written in detail for Night Float.

Schedule Changes:

If you require any schedule changes, such as an approved PTO or would like to switch shifts with another co-resident, the Chiefs must be informed via email using the PTO request form.

To make this process streamlined for all internal medicine residents, you will need to fill out the following form when all approved, changes in the schedule will not be approved until the request is submitted:

AMBULATORY CARE POLICY

The Internal Medicine Residency Program at MHS offers a 4-1 block rotation schedule, which means 1 week of continuity clinic after 4 weeks of inpatient service. Continuity of care, along with comprehensive care and coordinated care, defines the general internist's practice. An appreciation of the importance of the physician-patient relationship, diagnostic skills, patient advocacy, case management, professionalism, and continuity permeates the general medicine ambulatory experience. Particular emphasis is given to disease prevention and health promotion. The clinic operation also provides the opportunity to learn efficiency and economy in the out-patient setting along with the economics of medical practice.

The goals of the teaching program are to expand each resident's knowledge of the principles of continuity of care as it applies to the medical care of their patients. The competencies of the general medicine ambulatory experience include the knowledge, skills, and attitudes that make the difference between the provision of episodic, fragmented, and occasionally ineffective care, and care that typifies quality general internal medicine practice with an emphasis on continuity of care. The principal educational goals for the ambulatory program are listed in the Ambulatory Care Curriculum for each of the six ACGME competencies and entrusted professional activities, each being mapped to the ABIM Milestones

Purpose

The purpose of the Internal Medicine Continuity Clinic is to provide high quality, compassionate care to outpatients while providing an outstanding education experience for residents and students. By the end of the first year of the rotation residents should be able to perform a focused history and physical exams for common ambulatory complaints, counsel patients regarding preventive care, formulate management plans for common acute and chronic disease processes, and use the electronic health record effectively to document and manage patient care. In the second and third year residents should be expanding their knowledge in these areas and should be able to formulate management plans for patients with complex, multiple diseases. They should be familiar with the system resources and be able to effectively manage the patients' care using these resources. Effective communication with primary care physicians, consultants or referring physicians will be expected. All service activities will be in accordance with the ACGME Residency Committee Essential Requirements for Internal Medicine Residency training programs, policies and procedures of Trinity Hospital.

Patient Care Benchmark

- 1 attending provider oversees about 2-4 residents each session. Attending providers can average 12-20 visits per half-day session.
- Resident are required to complete note on day of service, prior to leaving clinic
 - Attending must review, dictate, and sign off on everything – including labs & controlled substances
- In order to meet requirements to graduate the program, and for residents to be successful in their own practices after graduation, GME has set the below benchmarks for visits per session based on national standards
 - PGY 1 should have 2 to 3 scheduled visits in the first 6 months of training. Thereafter, they should have 4 scheduled visits per session.
 - PGY 2 should have 6 scheduled visits per session.
 - PGY 3 should have 8 scheduled visits per session.

The expectation is that residents should be able to see incrementally more patients by 2nd and 3rd year. This is to prepare for life after residency.

One session for a resident or fellow equates to a half day, or 4 hours of clinic time. There are two sessions in each clinic day.

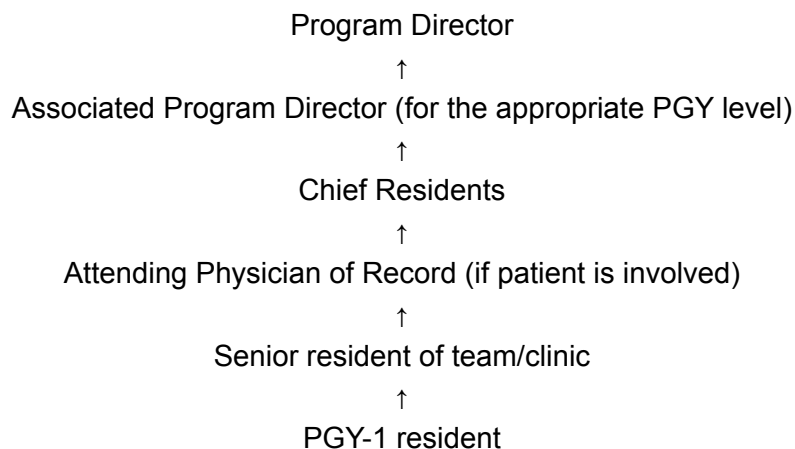
RESIDENT SUPERVISION & LINES OF AUTHORITY

The Program Director is responsible for supervision of all residents. Responsibility for specific supervision may be assigned to faculty supervising residents on various academic rotations.

As part of the training program, residents are given progressive responsibility for the care of patients and to act in a teaching capacity and provide supervision to less experienced residents and students. It is the decision of the Program Director, faculty, and attendings as to which activities residents are allowed to perform within the context of the assigned levels of responsibility. Documentation of supervision is by progress notes signed by attending physicians.

Lines of Authority

On occasion, a resident may need advice regarding patient management or administrative issues. In such cases, the resident should generally seek advice from the supervisor next-in-line above him/her. The following hierarchy of authority should be used in these situations:



Should disagreements between residents arise, the Chief Resident should be contacted immediately. All inquiries are completely confidential and will be dealt with to ensure a solution for both parties. As a program, we should all be one team with an ultimate goal of excellent patient care.

Each intern will also be assigned a senior resident as well as an attending physician as a mentor, whom they will meet up with privately every quarter to discuss any concerns and possible improvements in their learning.

Attending Physician Teaching and Supervision Responsibilities

Conduct teaching or management/teaching rounds with the house staff at least seven hours per week and assist with any problems that arise. These rounds should not interfere with morning reports or noon conferences and must include case presentations, interpretation of data, discussion of pathophysiology, differential diagnosis, management, use of technology, use of best evidence and patient values in decision making, disease prevention, and bedside teaching.

Attending physicians should also:

- Review the rotation curriculum with the housestaff at the beginning of the rotation. The curriculum is available online.
- Supervise and teach team members. Review and critique medical students' and house-staff's history and physicals, daily progress notes, and oral presentations.
- Accept medical responsibility for the care of patients assigned to the service. Write a brief admit note on all patients within 24 hours of admission documenting that the patient has been examined, the housestaff documentation has been reviewed, and recommending any changes in assessment or management. It is strongly encouraged to complete daily progress notes.
- Be available by phone at all times to assist housestaff and be available in person if requested. Attempt to be present during procedures.
- Provide feedback to both students and house staff at the end of the rotation. If a team member's performance is unsatisfactory, it is the duty of the attending physician to notify the student or housestaff officer as soon as a problem is noticed to provide the team member ample opportunity for improvement.